BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Full blood count, kidney and liver tests, blood pressure and an electrocardiogram (ECG) if at risk of heart-rhythm changes. Review recent cardiovascular disease, all medicines, bone health and falls risk.	Repeat blood count, kidney and liver tests on your agreed schedule; blood pressure and ECG as clinically indicated. The National Cancer Control Programme (NCCP) regimen does not give a universal blood-test or scan interval. Monitor more closely with kidney or liver impairment.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take the morning and evening doses with food; swallow tablets whole with water. Take a missed dose when remembered before the next scheduled dose, then return to the normal schedule. Do not take two doses together or double the dose. Confirm any prescribed reduction with your team.	Keep a current dose and symptom record.	Ask pharmacy to check all medicines, especially cholesterol medicines such as rosuvastatin and enzyme-inducing medicines or herbal products. Report jaundice, dark urine, severe tiredness, chest symptoms or a marked change in wellbeing promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	NCCP selection includes PSA at least 2 ng/mL, PSA doubling time no more than 10 months and testosterone below 1.7 nmol/L. PSA, symptoms and appropriate scans are interpreted together.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00693 v2: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, blood pressure and an ECG if at risk of heart-rhythm changes. Review recent cardiovascular disease, all medicines, bone health and falls risk.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Repeat blood count, kidney and liver tests on your agreed schedule; blood pressure and ECG as clinically indicated. The NCCP regimen does not give a universal blood-test or scan interval. Monitor more closely with kidney or liver impairment.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

NCCP selection includes PSA at least 2 ng/mL, PSA doubling time no more than 10 months and testosterone below 1.7 nmol/L. PSA, symptoms and appropriate scans are interpreted together.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Darolutamide: high-risk nonmetastatic castration-resistant prostate cancer: treatment and checks

Castration-resistant prostate cancer with no metastases detected on conventional imaging and a high risk of developing metastases, as defined in NCCP regimen 00693.

Darolutamide 600 mg twice daily with food, continuously while helping and tolerated. Continue ADT unless surgically castrated. The team may prescribe 300 mg twice daily for selected organ impairment or toxicity.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, blood pressure and an ECG if at risk of heart-rhythm changes. Review recent cardiovascular disease, all medicines, bone health and falls risk.	Repeat blood count, kidney and liver tests on your agreed schedule; blood pressure and ECG as clinically indicated. The NCCP regimen does not give a universal blood-test or scan interval. Monitor more closely with kidney or liver impairment.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take the morning and evening doses with food; swallow tablets whole with water. Take a missed dose when remembered before the next scheduled dose, then return to the normal schedule. Do not take two doses together or double the dose. Confirm any prescribed reduction with your team.	Keep a current dose and symptom record.	Ask pharmacy to check all medicines, especially cholesterol medicines such as rosuvastatin and enzyme-inducing medicines or herbal products. Report jaundice, dark urine, severe tiredness, chest symptoms or a marked change in wellbeing promptly.	Keep follow-up and recovery checks even after the last dose.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests NCCP	NCCP selection includes PSA at least 2 ng/mL, PSA doubling time no more than 10 months and testosterone below 1.7 nmol/L. PSA, symptoms and appropriate scans are interpreted together.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1

Treatment and review

Twice daily with food plus ongoing ADT

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Repeat blood count, kidney and liver tests on your agreed schedule; blood pressure and ECG as clinically indicated. The NCCP regimen does not give a universal blood-test or scan interval. Monitor more closely with kidney or liver impairment.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.51